



HEALTH INSURANCE CLAIM FORM

APPROVED BY NATIONAL UNIFORM CLAIM COMMITTEE 02/12

GEICO
NY PIP
P.O. BOX 9507
FREDERICKSBURG, VA 22403

1. medicare ☐ (medicare #) ☐ medicare ☐ (medicaid #) ☐ tricare ☐ ID#/DoD# ☐ champva ☐ (Member ID#) ☐ group health plan (SSN or ID) ☐ fecla bkl lung (SSN) ☐ other ☒ (id)		1a. insured's i.d. number (for program in item 1) 032 139 135 0101 023	
2. PATIENT'S NAME (Last Name, First Name, Middle Initial) ALISHA K MEERTENS		3. PATIENT'S BIRTH DATE MM DD YY 04 03 1981 SEX M ☐ F ☒	
5. PATIENT'S ADDRESS (No., Street) 123 Beach 61st St		6. PATIENT RELATIONSHIP TO INSURED Self ☐ Spouse ☐ Child ☐ Other ☐	
CITY Queens		CITY	
STATE NY		STATE	
ZIP CODE 11692		ZIP CODE	
TELEPHONE (Include Area Code) (917) 674-6903		TELEPHONE (Include Area Code)	
9. OTHER INSURED'S NAME (Last Name, First Name, Middle Initial)		10. IS PATIENT'S CONDITION RELATED TO:	
a. OTHER INSURED'S POLICY OR GROUP NUMBER		a. EMPLOYMENT? (Current or Previous) ☐ YES ☒ NO	
b. RESERVED FOR NUCC USE		b. AUTO ACCIDENT? ☒ YES ☐ NO	
c. RESERVED FOR NUCC USE		c. OTHER ACCIDENT? ☐ YES ☒ NO	
d. INSURANCE PLAN NAME OR PROGRAM NAME		10d. CLAIM CODES (Designated by NUCC)	
11. INSURED'S POLICY GROUP OR FECA NUMBER 4093414870		11. INSURED'S DATE OF BIRTH MM DD YY SEX M ☐ F ☐	
12. PATIENT'S OR AUTHORIZED PERSON'S SIGNATURE I authorize the release of any medical or other information necessary to process this claim. I also request payment of government benefits either to myself or to the party who accepts assignment below. SIGNED SIGNATURE ON FILE DATE 12/15/2011		13. INSURED'S OR AUTHORIZED PERSON'S SIGNATURE I authorize payment of medical benefits to the undersigned physician or supplier for services described below. SIGNED SIGNATURE ON FILE	
14. DATE OF CURRENT ILLNESS, INJURY, or PREGNANCY (LMP) MM DD YY 10 12 2011 QUAL.		15. OTHER DATE QUAL. MM DD YY	
17. NAME OF REFERRING PROVIDER OR OTHER SOURCE Igor Zakharov		18. HOSPITALIZATION DATES RELATED TO CURRENT SERVICES FROM MM DD YY TO MM DD YY	
19. ADDITIONAL CLAIM INFORMATION (Designated by NUCC)		20. OUTSIDE LAB? ☐ YES ☒ NO \$ CHARGES	
21. DIAGNOSIS OR NATURE OF ILLNESS OR INJURY (Relate Items 1, 2, 3 or 4 to Item 24E by Line) 1. 723.1 2. 722.0 3. 839.00 4. 722.6 5. 737.30 6. 737.9 7. 724.2 8. 722.10		22. RESUBMISSION CODE ORIGINAL REF. NO. 23. PRIOR AUTHORIZATION NUMBER	
24. A. DATE(S) OF SERVICE From MM DD YY To MM DD YY 11 17 11 11 17 11		B. PLACE OF SERVICE 11	
C. EMG		D. PROCEDURES, SERVICES OR SUPPLIES (Explain Unusual Circumstances) CPT/HCPCS MODIFIER 72141	
E. DIAGNOSIS POINTER 1 2 3 4 5 6 7 8		F. \$ CHARGES \$879.73	
G. DAYS OR UNITS 1		H. EPSDT Family Plan N	
I. ID. QUAL. NPI		J. RENDERING PROVIDER ID. # 183333	
25. FEDERAL TAX I.D. NUMBER 26-1323136		26. PATIENT'S ACCOUNT NO. 493	
27. ACCEPT ASSIGNMENT? (For govt claims, see back) ☐ YES ☒ NO		28. TOTAL CHARGE \$1,791.73	
29. AMOUNT PAID \$		29. AMOUNT PAID \$1,791.73	
30. Rsvd for NUCC			
31. SIGNATURE OF PHYSICIAN OR SUPPLIER INCLUDING DEGREES OR CREDENTIALS (I certify that the statement on the reverse apply to this bill and are made a part thereof) WINTER W STEVEN W. M.D. SIGNED DATE 12/15/2011		32. SERVICE FACILITY LOCATION INFORMATION STELTON RADIOLOGY CORP 1849 Utica Avenue Brooklyn, NY 11234	
33. BILLING PROVIDER INFO & PHONE # STELTON RADIOLOGY CORP 1849 Utica Avenue Brooklyn, NY 11234 (718) 676-5595		a. 183333 b.	



STEVEN W. WINTER, M.D. P.C.

1849 Utica Avenue Brooklyn, NY, 11234
Phone:(718) 676-5595 Fax:(718) 709-7499

PATIENT NAME: Alisha Meertens

DATE OF SERVICE: 11/17/2011

REFERRING PHYSICIAN: Igor Zakharov

MRI Cervical spine:

Multi-planar, multi-echo images through the cervical spine were performed.

There is reversal of the cervical curvature having its apex at the C5/6 level.

C2/3 demonstrates a left posterior extruded disc herniation that impresses the left ventral thecal sac and extends into the left anterior recess where the left cord is impressed upon.

Subligamentous posterior disc herniextension.

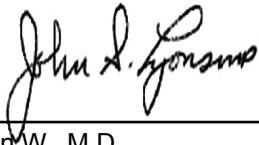
At C5/6 is a subligamentous disc herniation that has a left posterior component that abuts the left ventral cord and encroaches into the left anterior recess.

A left focal subligamentous disc herniation and radial annular tear impresses on the left ventral thecal sac at C6/7. ation at C3/4 is impressing on the ventral cord. A subligamentous disc herniation left posteriorly at C4/5 impresses on the left ventral cord and is associated with anterior disc

Otherwise, there is no evidence of discogenic disease, disc bulge or disc herniation. There is no compromise of the cervical cord, thecal sac or neural foramina. There is no bony destructive lesion or paravertebral soft tissue mass.

1. C2/3 demonstrates a left posterior extruded disc herniation that impresses the left ventral thecal sac and extends into the left anterior recess where the left cord is impressed upon.
2. Subligamentous posterior disc herniextension.
3. At C5/6 is a subligamentous disc herniation that has a left posterior component that abuts the left ventral cord and encroaches into the left anterior recess.
4. A left focal subligamentous disc herniation and radial annular tear impresses on the left ventral thecal sac at C6/7. ation at C3/4 is impressing on the ventral cord. A subligamentous disc herniation left posteriorly at C4/5 impresses on the left ventral cord and is associated with anterior disc
5. C2/3 demonstrates a left posterior extruded disc herniation that impresses the left ventral thecal sac and extends into the left anterior recess where the left cord is impressed upon.
6. Subligamentous posterior disc herniextension.
7. At C5/6 is a subligamentous disc herniation that has a left posterior component that abuts the left ventral cord and encroaches into the left anterior recess.
8. A left focal subligamentous disc herniation and radial annular tear impresses on the left ventral thecal sac at C6/7. ation at C3/4 is impressing on the ventral cord. A subligamentous disc herniation left posteriorly at C4/5 impresses on the left ventral cord and is associated with anterior disc.
9. There is reversal of the cervical curvature having its apex at the C5/6 level.

Thank you for the courtesy of this consultation.

A handwritten signature in black ink, appearing to read "John L. Forns". The signature is written in a cursive style with a large, stylized "J" and "F".

Winter Steven W., M.D.

Diplomate American Board of Radiology Board Of Education 1234567890.

Diplomate American Board of Radiology.



STEVEN W. WINTER, M.D. P.C.

1849 Utica Avenue Brooklyn, NY, 11234
Phone:(718) 676-5595 Fax:(718) 709-7499

PATIENT NAME: Alisha Meertens

DATE OF SERVICE: 11/17/2011

REFERRING PHYSICIAN: Igor Zakharov

MRI Lumbar spine:

Multi-planar, multi-echo images through the lumbar spine were performed.

There is a left convexity of the lumbar curvature.

At the L5/S1 level there is a subligamentous disc herniation with a 1 mm retrolisthesis and disc hydration loss that extends into the epidural fat and nearly abuts the ventral thecal sac and exiting S1 nerve roots from the thecal sac. Peripheral components of the disc are nearly abutting the L5 nerve roots in the foramen.

L4/5 demonstrates subligamentous disc bulging.

Otherwise, there is no evidence of discogenic disease, disc bulge, disc herniation or compromise of the thecal sac, lateral recesses or neural foramina. There is no conus medullaris lesion, fracture, bony destructive lesion or paravertebral soft tissue mass otherwise demonstrated.

- 1. At the L5/S1 level there is a subligamentous disc herniation with a 1 mm retrolisthesis and disc hydration loss that extends into the epidural fat and nearly abuts the ventral thecal sac and exiting S1 nerve roots from the thecal sac. Peripheral components of the disc are nearly abutting the L5 nerve roots in the foramen.**
- 2. L4/5 demonstrates subligamentous disc bulging.**
- 3. Left convexity of the lumbar curvature.**

Thank you for the courtesy of this consultation.

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